



Bariatric Management Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	All disciplines

2. Purpose

The Armada Kalamunda Group (AKG) aims to address the needs of bariatric individuals in a safe and dignified manner whilst minimising the risk of injuries to staff, patients and visitors through provision of suitable resources, including the promotion of appropriate patient handling equipment, processes and techniques.

3. Equipment

Standard adult beds have a safe working limit (SWL) of 250kg (except Mental Health – 150kg). All wards have a list of their own bariatric equipment; wards should liaise with other wards/units should they need to borrow equipment.

If the relevant equipment is not available within the hospital, bariatric equipment may be rented (see HSS contracts [Hospital Bedding Equipment](#) and [Patient Aids and Appliances](#)). The Nurse Unit Manager is responsible for arranging any rental equipment in collaboration with the relevant Allied Health staff.

There are operational areas within AKG where there are weight limitations due to access or special equipment requirements. The table below outlines where these areas are and the access and special equipment requirements:

Operational area	Equipment requirements
Medical Imaging	X-ray – 300kg general room table / 135kg screening room CT scanner – 205kg Ultrasound couches – 200kg MRI scanner – 150kg stretcher / 250kg walking if independent with transfer <i>Note there are girth limitations for MRI and CT Scanners</i>
Operating Theatres (AHS)	270kg for operating tables
Same Day Unit	Trolley – 300kg
Hearts West	Discuss patients over 120kg. Depending on mobility, the service may be performed on the ward
Kalamunda Hospital	230kg due to St John Ambulance emergency transfer ability

Operational area	Equipment requirements
ICU	Hill Rom beds 227 kg Howard Wright beds 249.45 kg Bariatric bed (Citadel plus) 454 kg Hoist 320 kg

4. Procedure

4.1 Bariatric Risk Assessment

All patients who weigh ≥ 120 kg, or with a body mass index (BMI) >35 will require a Bariatric Patient Initial Risk Assessment form (AKMR 111.2.1) to be completed as per the requirements below:

- Planned (elective) admissions – Part A at Preadmission Clinic
- Unplanned (emergency) admissions – Part A in the Emergency Department
- Direct admissions – Part A and B on admission to the ward.

The control measures (Part B) for planned and unplanned admission will be completed on admission to the ward.

Planned admissions that are day cases and will be discharged from the Same Day Unit do not require the AKMR 111.2.1 to be completed as this information is recorded within the Theatre Management System (TMS).

All patients regardless of their weight or BMI will be provided with emergency care in the Emergency Department.

4.2 Safe Working Limits

Managers need to be aware of the safe working limits of the standard equipment for their wards/units including:

- Beds/trolleys
- Armchairs
- Shower chairs/stools
- Wheelchairs
- Commodes
- Over toilet frames
- Hoists

4.3 Physical Environment

On occasion, patients utilising bariatric equipment will require a larger area than a standard hospital room. Armadale Health Service has the following designated larger rooms:

- Rooms 39 and 40 on RAC – ceiling room to bathroom hoists (SWL 350kg)
- Rooms 29 and 36 on RAC – ceiling hoists room only (SWL 250kg)
- Rooms 37 and 38 on RAC can be converted to one room with a room to bathroom ceiling hoist (SWL 350kg).
- Kalamunda Hospital:
 - Afford patients a room appropriate for allowing correct manual handling techniques.

If these rooms are not suitable, adjacent bed closures in other areas can be considered after discussion with the Hospital Nurse Manager.

4.4 Manual Handling

Patients should be encouraged to maintain independence of movement as much as possible, thus reducing manual handling requirements.

Staff should utilise appropriate manual handling techniques and equipment in accordance with the [Manual Handling Patient – Assessment Tool](#) and the [Safe Handling of Loads – Observation Checklist](#) as learnt in their patient handling mandatory training, and ensure they have the correct equipment in place.

4.5 Special Considerations

4.5.1 Pregnancy

Pregnancy in women with an increased BMI can lead to increased complications, and may require tertiary level care – see the [Obesity in Pregnancy Guideline \(OBS-GUI-0308-14\)](#).

4.5.2 Perioperative patients

Patients who weigh more than 100kg may need particular attention during the perioperative period. Attending staff are to assess the dimensions and balance of patient supporting devices such as operating tables, trolleys and beds.

Staff should ensure avoidance of body overhand and lateral compression via the use of bed extensions.

When determining the requirement for staff to handle a bariatric patient the following should be considered:

- <90kg three staff members required
- >100kg minimum of 4 staff members required plus hover mat.

The following restrictions are based on the ergonomic aspects of the facility and SWLs of equipment available in the Perioperative Suite:

- Operating table – 270kg
- Trolley transfers – 240kg – with the patient in balance over the centre of the trolley.
- Extra care with additional personnel and supports may be required if the side mechanism is used for more than 5 degree tilt laterally or longitudinally. Hydraulics not guaranteed for weights exceeding 135kg.

In emergencies these guidelines may be varied by medical and nursing staff for the most appropriate clinical management at the time.

It is recommended that patients identified by the guidelines as “at risk” will be referred to the anaesthetist as early as possible pre-operatively for final assessment.

5. Transport Options

The following transport options may be considered for patients unable to travel in their own transport, or by public transport:

- Maxi Taxi – up to a total weight of 350kg (including wheelchair).
- Royal Flying Doctor Service – up to 166kg.
- Helicopter – up to 159kg on the Ferno Litter and 272kg on the Helicopter Winching Basket.
- St John Ambulance – Up to 230kg in a standard ambulance with a Ferno 50E stretcher, or up to 500kg in the complex patient ambulance transport vehicle (advise when booking).

These SWLs should be checked at the time of booking as they are subject to change.

6. End of Life Care

- Deceased patients between 100–120kg should be transferred to the mortuary on the patient's bed with two staff to move the patient into the mortuary fridge.
- Deceased patients between 120–220kg must be assessed to ensure the girth of the patient can be accommodated within the mortuary fridge. These patients should be transferred to the mortuary on the patient's bed with a minimum four staff in attendance to move the patient into the mortuary fridge.
- Deceased patients >220kg or whose girth cannot be accommodated within the mortuary fridge must be collected directly from the ward by the funeral directors – see the [Mortuary Patients Procedure \(AKG-PRO-0269-17\)](#).

7. Bariatric Visitors

Consideration must be given by AKG staff as how to accommodate a bariatric visitor such as a parent wishing to stay overnight with their sick child. This may include:

- Provision of bed, chair and personal hygiene facilities will need to be addressed in terms of access, availability and safe working loads.
- Generally, wheelchairs able to be utilised by visitors have a SWL of 100kg. There are two wheelchairs available at the main entrance with a SWL of 150kg, and one with a SWL 180kg. If the visitor requires one of these wheelchairs staff are not permitted to push the visitor in the chair - this needs to be the responsibility of the family members.

8. Legislative Context

The following documents are required to give effect to this procedure:

- *Work Health and Safety (General) Regulations (WA) 2022*

- *Work Health and Safety Regulations 2020*
- EMHS [Work Health and Safety: Manual Tasks Policy \(EMHS: 04\)](#)
- EMHS [Mandatory Training Policy \(EMHS: 51\)](#)

9. Supporting Information

The following documents support the implementation of this procedure:

- [Mortuary Patients Procedure \(AKG-PRO-0269-17\)](#)
- [Obesity in Pregnancy Guideline \(OBS-GUI-0308-14\)](#)
- [WA Health Morbid Obesity Model of Care](#)

10. Compliance, Monitoring and Evaluation

Compliance against this procedure will be evaluated and monitored by the Clinical Safety and Quality Committee.

Compliance monitoring methods include:

- Delays in admission / treatment related to a bariatric matter / patient.
- Audits of existing bariatric equipment stock at each site.
- Staff incident and hazard reports related to bariatric manual tasks.
- Clinical incident reports relating to the management of the bariatric patient.
- Complaints related to a bariatric issue.
- Evaluation of the effectiveness of Bariatric Management Plans.
- Audit to determine whether individuals who accessed inpatient and outpatient services had their BMI score measured.

11. Relevant Standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.15 Diversity and high-risk groups
- Standard 5 – 5.10 Screening of risk
- Standard 5 – Action 5.11 Clinical assessment

12. Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Nurse Coordinator Surgical & Acute
Department: Perioperative Management
Email: shane.hastings@health.wa.gov.au

13. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 2.0	31/08/2022	31/08/2025	Bariatric Patient Initial Risk Assessment added as an appendix

14. Authorisation

This procedure has been authorised and issued by Director Clinical Service.

Authorised by	Donn John, A/Director Nursing and Midwifery
Authorisation date	06/09/2022
Published date	07/09/2022

Appendix 1: Bariatric Patient Initial Risk Assessment

Appendix 1

Please use ID label or block print

ARMADALE KALAMUNDA GROUP BARIATRIC PATIENT INITIAL RISK ASSESSMENT ≥120Kg and/or BMI >35	Family Name:	UMRN:
	Given Names:	
	Gender:	DOB:

Patient Weight:	Patient Height:	BMI: wt(kg)/ Ht(m) ²
Reason for Admission:		
Date of Admission:		

PART A

INITIAL MOBILITY SCREEN	RESPONSE (tick)	
	Yes	No
Bed Mobility Able to independently roll side to side, move up/down the bed, move supine to sitting on edge of bed?	☐	☐ Refer Controls A to D
Transfers Able to transfer independently from chair to chair? Able to transfer independently from bed to chair?	☐ ☐	☐ ☐ Refer Controls A to D
Ambulation Able to weightbear and walk independently/minimal assist only?	☐	☐ Refer Control A
Is the patients BMI >35	☐ Control E	☐

DIAGNOSTIC REQUIREMENTS	Yes	No
Is there an imaging/diagnostic table that caters for the weight of this patient?	☐	☐
Ultrasound - SWL 200kg CT table – SWL 205kg X-ray table –SWL 300kg (general room table) MRI – SWL 150kg (stretcher patients)		Consult with Perth Radiological Clinic if imaging required.

Staff name (print clearly) _____ Date _____

Complete part A in the Emergency Department for emergency admissions, Preadmission Clinic for elective admissions, or on the ward for direct admissions.

May 2017

AKMR 111.2.1 BARIATRIC PATIENT INITIAL RISK ASSESSMENT ≥120 KG AND/OR BMI >35

Please use ID label or block print

ARMADALE KALAMUNDA GROUP BARIATRIC PATIENT INITIAL RISK ASSESSMENT ≥120Kg	Family Name:	UMRN:
	Given Names:	
	Gender:	DOB:

PART B – to be completed on admission to the ward

CONTROL MEASURES	Yes	No
A. Referral to Physio/OT for mobility plan/review		
B. Equipment required for safe management. Can this patient use your ward's standard equipment?		
Bed		
Chair		
Wheelchair		
Commode		
Shower Chair		
Over Toilet Frame		
Slide Sheet		
Hover Mat		
Hoist		
Nimbus Mattress		
Bariatric Walking Aids		
C. Functional space required Does the patient require a large room to accommodate the required equipment and safe working practices of staff.		
D. Additional staff required.		
E. Referral to Dietitian		

If patient transferred to another ward, equipment must go with them. Any problems with equipment or availability advise Occupational Therapist/ Nurse Manager.

Staff name (print clearly) _____ **Date** _____